

Kaiser Permanente Southern California

Provider Practice Alert

Patient Education Requirements: Blood Product Administration



Situation:

For non-emergent blood transfusions ONLY, the law requires Providers (physicians, physician assistants, midwives, nurse practitioners, and nurse anesthetists) to both inform patient/significant other, or parent and provide them with a standardized written summary detailing the following: blood product **treatment**, potential **benefits**, **risks** of the procedure, and **alternatives**. The provision of this education and printed materials must be documented in the EHR.

Background

On January 1, 1990, the Paul Gann Blood Safety Act became law (Paul Gann Blood Safety Act, H&S § 1645). As described above, this mandate requires Providers to inform patient/significant other, or parent of the risks and benefits for blood product transfusions and provide a written summary of those instructions: “A Patient’s Guide To Blood Transfusion”.

English and Spanish brochures (“A Patient’s Guide To Blood Transfusion”) are prepared by the California Department of Health Services and are located within the appendices of the latest Blood Administration and Transfusion Reaction policy. Latest versions of “A Patient’s Guide To Blood Transfusion” can be found online (https://www.mbc.ca.gov/Publications/Brochures/Blood_Transfusions.aspx).

During a recent Joint Commission survey a medical center was cited for non-adherence to the Paul Gann Act as evidence by missing documentation within the Electronic Health Record (EHR).

Assessment

Variability exists for the consistent application of the Paul Gann Act.

Recommendation:

Effective December 3, 2019, SCAL will disseminate a Regional policy (Blood Administration and Transfusion Reaction) to all KPSC medical centers. This policy aligns with the Paul Gann Act and contains procedural descriptions and appendices to support the consistent application of the patient education and documentation mandates.

Overview

- Provider obtains informed consent from the patient/significant other, or parent regarding risks, benefits, and alternatives to treatment.
- “A Patient’s Guide To Blood Transfusion” (standardized written summary) brochure outlining the treatment, potential benefits, risks, and alternatives is provided to the patient/significant other, or parent.
- Delivery of the brochure and education is documented in the EHR.
- The education and delivery of the brochure must be completed by the provider.

A SmartText in KP HealthConnect (KPHC) is readily available for providers to capture their informed consent conversations and delivery of the required brochure (see attached).

Medical Centers may consider adding these Paul Gann requirements to existing blood transfusion audits to monitor compliance with the law and policy.

“A Patient’s Guide to Blood Transfusion”

(approved standardized written summary: 2-sided trifold brochure)

References:

- Circular of Information for the Use of Human Blood and Blood Components. AABB. Nov 2013 (revised April 2014)
- AABB Technical Manual. 18th Edition.

This brochure is provided as a source of information and is not considered a replacement for the Informed Consent process prior to the transfusion of blood.

A Patient’s Guide to Blood Transfusion

California
Department of Public Health

June 2018

This brochure was developed by the California Department of Public Health, Laboratory Field Services (850 Marina Bay Parkway, Richmond, CA 94804)

In partnership with the Medical Technical Advisory Committee of the Blood Centers of California.

For information about brochure contents, please call Laboratory Field Services: (213) 620-6574

Distributed by the Medical Board of California

To place an order for this brochure, please FAX your request to: (916) 263-2497

This information may be obtained electronically at:
http://www.mbc.ca.gov/Publications/Brochures/Blood_Transfusions.aspx

Revised 06/2018

This document provides written information regarding the benefits, risks, and alternatives of transfusion of blood products (including red blood cells, plasma, platelets, or others) collected from the patient (autologous) or another person. This material serves as a supplement to the discussion you have with your physician. It is important that you fully understand this information, so please read this document thoroughly. If you have any questions regarding the procedure, ask your physician prior to consenting to receive a transfusion.

■ Information about the treatment
Transfusions of blood products are provided to increase the amount of blood components in your body when they are below a reasonable level for your health. The transfusion may be made up of red blood cells, plasma, platelets or other specialized products made from blood. Your physician will decide on the right amount and type of blood product based on your medical condition or diagnosis.

■ Potential benefits of the treatment
Transfusion of blood products may be necessary to correct low levels of blood components in your body, and may also make you feel better. In some cases, failure to receive transfusion(s) may result in death.

■ Risks of the treatment
Known risks of this treatment include, but are not limited to:

- Irritation, pain, or infection at the needle site
- Temporary reaction such as a fever, chills, or skin rashes.

Other rare but more serious complications include severe allergic reactions, heart failure due to fluid overload, acute pulmonary edema (fluid leaking into the lungs), hemolysis (destruction of red blood cells), shock, or death.

Transfusion of blood products carries a very small risk of transmission of infectious diseases such as HIV (about 1 in 1.5 million), Hepatitis C (about 1 in 1.2 million), and Hepatitis B (about 1 in 1 million). Other significant infections may also be transmitted by transfusion, but overall this risk is low.

■ Treatment Options/Alternatives
If you need blood you have several options. Most patients requiring transfusion receive blood products donated by volunteer community donors. These donors are extensively screened about their health history and undergo numerous blood tests as mandated by state and federal regulations in order to ensure the safest possible blood supply. Alternatives to transfusion with blood products from volunteer community donors include:

- **Pre-operative autologous donation** (using your own previously donated blood), see below for more information
- **Directed donation** (blood donated by people who you have asked to donate for you), see below for more information
- **Intra-operative autologous transfusion/Hemodilution** (collecting your own blood during surgery to be given back to you)

■ Medications (certain medications may increase blood volume prior to surgery or reduce active bleeding to lessen the need for transfusion)

These options may be available only if your health, time, and procedure permit. They may not be available at all locations or for all patients. You may also choose not to receive blood transfusion; however this decision may hold life-threatening consequences.

Pre-operative autologous donation is not appropriate for all patients. Autologous donation involves collecting your own blood prior to a planned surgery for storage in the hospital blood bank. It is important to discuss with your physician if it is safe for you to donate and the likelihood of needing a transfusion based on your surgery and current transfusion guidelines. Receiving your own blood may reduce, but will not eliminate, the risk of transfusion-related complications. Insurance company policies may vary regarding reimbursement for this service. Overall, although autologous donation is an option to consider for those who qualify, the number of autologous donations in the United States has significantly decreased in the last few decades mainly due to major advances in blood safety and efforts to decrease unnecessary blood transfusions.

Directed donation refers to blood collected from “directed donors” who are donating blood for a specific patient by request. Directed donors are often family and friends of the patient. Directed donors go through the same qualification process as volunteer donors. Directed donations are not considered to be safer than the general blood supply.

SmartText (“paul gann act”)

SmartText Lookup

Matches

- ★ PAUL GANN BLOOD SAFETY ACT GEN...

Preview

PHYSICIAN’S STATEMENT CONCERNING TRANSFUSION INFORMATION
(Paul Gann Blood Safety Act. Health and Safety Code #1645)

I have informed the patient by means of a standardized written summary prepared by the California Department of Health Services concerning the positive and negative aspects of receiving autologous blood and directed and nondirected homologous blood from volunteers. Except if the patient has waived it, or there are medical contraindications or a life threatening emergency, I have allowed adequate time prior to the procedure for pre-donation to occur.

Electronically signed by:

11/7/2019
11:34 AM

I

☐ Favorites Only

✓ Accept
✗ Cancel